

إسم المريض رابعياً : فاطمة عبدالعزيز محمد عبدالنبي

العنوان : المطريه القايره

تاريخ تسجيل الإستمارة : 26-09-2026

العمر(تاريخ الميلاد) : 18-07-1965 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

**قسم العلاج :
الاورام**

خطة العلاج : علاج مبادرة سرطان الثدي مثبط CDK4/6 و فلفسترات [14469]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: فاطمة عبدالعزيز محمد عبد الله National ID: 26507180101824	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي CDK4/6 inhibitor and fulvestrant

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

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شارع طلوعة أول شارع الهرم بجوار ناري ، الجيزة ، مصر 4

شارع طلوعة أول شارع الهرم بجوار ناري ، الجيزة ، مصر 4

1 Giza

Giza, Egypt

Patient Name: FATMA ABD EL AZIZ MOHAMMED A	Gender: Female
MRN: 10124244453	Ordered By: Eman Sobhy Adrawos Mahgoub
DOB: 18/07/1965 Age: 60 Y 9 M 1 D	Order ID: 3806397

CT CHEST

Procedure Date: 14/04/2026 09:31:00 AM

Report:

Report:

Multislice NON-contrast CT scan of the chest, abdomen and pelvis revealed:

* Status post follow up showing for a known case of metastatic breast cancer.

* As correlation was made with the previous non-contrast CT study dated 14/01/2026.

FINDINGS:

(I) Chest:

- Stationary course regarding:
 1. Right breast UOQ mass is seen measuring about 32 x 19 mm along its maximum axial dimensions attached to the overlying skin with still noted right breast parenchymal thickening and distortion as well as skin thickening.
 2. Right axillary lymph nodes, the largest measures 12 x 13 mm.
 3. Right internal mammary lymph node measuring 5 mm in diameter.
 4. Right supraclavicular and posterior triangle prominent lymph nodes are seen, the largest measures about 5 mm
 5. Few millimetric left supraclavicular lymph nodes.
 6. Multiple prominent left axillary (level I and II) lymph nodes.
 7. Few mediastinal lymph nodes are seen measuring up to 11 x 8 mm at precarinal group.
- Still, no sizable pulmonary nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- Still noted left upper lobe tiny calcified nodule.
- No pleural or pericardial collections seen.
- Average cardiac size.
- Still noted bilateral thyroid lobe hypodense nodules with scattered calcific foci.
- Small hiatus hernia.

(II) Abdomen and pelvis:

- Mildly enlarged liver showing homogenous CT density with no focal lesions on non contrast basis.
- Calcular gall bladder.
- No intra or extra hepatic biliary radicles' dilatation.
- Bilateral upper calyceal tiny radiodense stones. No renal back pressure changes. Left renal cortical cyst measures about 2.5 cm.
- Otherwise, normal non-contrast CT appearance of the spleen, pancreas and suprarenal glands and the rest of the pelvic organs.
- Fibroid uterus seen.



مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

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شارع عطوة أول شارع الهرم بجوار داري ، الحيزة ، مصر 4

شارع عطوة أول شارع الهرم بجوار داري ، الحيزة ، مصر 4

1 Giza

Giza, Egypt

Patient Name: FATMA ABD EL AZIZ MOHAMMED A	Gender: Female	1
MRN: 10124244453	Ordered By: Eman Sobhy Adrawos Mahgoub	
DOB: 18/07/1965 Age: 60 Y 9 M 1 D	Order ID: 3806397	

- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

(III) The examined bone cuts show:

- Stationary course of previously noted predominately sclerotic osseous lesions noted at DV5, D8 pedicle, left iliac wing, left 8th rib posterior aspect and left 3rd rib anterior aspect.
- Stationary appearance of left femoral neck comminuted displaced fracture.
- Still noted ventral wedging with structural collapse of D12 vertebral body.
- Dorsolumbar spondylosis degenerative changes with double level L3 and L4 degenerative spondylolisthesis and evident subchondral degenerative changes of L3/4 opposing end plates.

OPINION:

- * Status post follow up showing for a known case of metastatic breast cancer.
- * As correlation was made with the previous non-contrast CT study dated 14/01/2026, the current study shows stationary course.

Approved by: Mennatallah Mohamed, MD

Date:15/04/2026 Time:14:01

Reported and electronically endorsed by:

Mennallah Mohamed Hanafy Hassan

Reported on: 15/04/2026 06:41:00 PM

بهيّة

مركز بهيّة للأورام والتصوير
وعلاج سرطانيات الثدي

مسح ذري

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1 Giza
Giza, Egypt

Patient Name: FATMA ABD EL AZIZ MOHAMMED A	Gender: Female
MRN: 10124244453	Ordered By: Aya Ahmed Kamal Mohamed
DOB: 18/07/1965 Age: 60 Y 6 M 0 D	Order ID: 3695204

مسح ذري

NM BONE SCAN

Procedure Date: 05/01/2026 09:33:00 AM

Report:

Report:
99mTc-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 23/7/2025, the current study revealed rather stationary course of the metastatic lesions at DV5/6, left 8th rib posteriorly, left sacro-iliac joint region, proximal left femur, manubrium sterni and left 3rd rib anteriorly as well as right 7th rib posteriorly.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

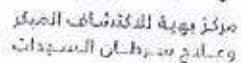
Rather stationary course of the metastatic osseous deposits.
No newly developed bone lesions.
Approved by: Prof. Huda Fathy, Nuclear Medicine Consultant
Date: 05/01/2026 Time: 15:27

Reported and electronically endorsed by:
Huda Fathy Elsayed Abdulhalim

Reported on: 05/01/2026 03:31:00 PM

Print Date: 18/01/2026 03:21:25 PM

Page: 1/1



Bahya

T 16052

F 10632

W. J. G. Smeets

1. *Journal of the American Medical Association*, 1997; 278: 1039-1044.

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1. 1000
2. 1000

Patient Name: FATMA ABD EL AZIZ MOHAMMED A	Gender: Female
MRN: 10124244453	Ordered By: Aya Ahmed Kamal Mohamed
DOB: 18/07/1965 Age: 60 Y 6 M 0 D	Order ID: 3695204

NM BONE SCAN

Procedure Date: 05/01/2026 09:33:00 AM

Report:

Report:

99mTc-MDP BONE SCAN

Desired Information: - skeletal survey.

Technique

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 23/7/2025, the current study revealed rather stationary course of the metastatic lesions at DV5/6, left 8th rib posteriorly, left sacro-iliac joint region, proximal left femur, manubrium sterni and left 3rd rib anteriorly as well as right 7th rib posteriorly. Degenerative changes are seen over thoracic lumbar spine.

Degenerative changes are seen over dorso-lumbar vertebrae. Arthritic changes are seen over the

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion

Rather stationary course of the metastatic osseous deposits.
No newly developed metastases.

No newly developed bone lesions.

Approved by: Prof. Huda Fathy, Nuclear Medicine Consultant
Date: 05/01/2026, Time: 16:23

Date:05/01/2026 Time:15:27

Reported and electronically endorsed by:

Huda Fathy Elsayed Abdulhalim

Reported on: 05/01/2026 03:31:00 PM

Print Date: 18/01/2026 03:21:25 PM

Page: 22

بهرية

مركز بهرية للاكتشاف المبكر وعلاج سرطان الثدي

بهرية

Bahrya

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شارع بهرية ١٠٠ شارع الثورة بصرى ، مصر ٤
شارع بهرية ١٠٠ شارع الثورة بصرى ، مصر ٤
1 OZA
Giza, Egypt

Patient Name:	FATMA ABD EL AZIZ MOHAMMED A	Gender:	Female
MRN:	1012424453	Ordered By:	Aya Ahmed Kamal Mohamed
DOB:	18/07/1965 Age: 60 Y 6 M 0 D	Order ID:	3695198

Procedure Date: 14/01/2026 10:25:00 AM CT ABDOMEN AND PELVIS

Report:

Report:

Multislice NON-contrast CT scan of the chest, abdomen and pelvis revealed:

* Status post follow up showing for a known case of metastatic breast cancer.

* As correlation was made with the previous non-contrast CT study dated 22/10/2025

FINDINGS:

(I) Chest:

- Mild regression as regard the previously noted:

1. Few right axillary lymph nodes at level I noted, the largest measures 8mm compared to 10 mm at the previous study.

2. Few mediastinal lymph nodes are seen measuring up to 11x8mm compared to 15x10 mm at the previous study.

- Marked regressive course regarding size and number of previously noted right supraclavicular and posterior triangle pathologically enlarged lymph nodes are seen, the largest currently measures about 5 mm compared to 16.5 x 12 mm.

- Stationary course regarding:

1. Right breast UOQ mass is seen measuring about 32 x 19 mm along its maximum axial dimensions attached to the overlying skin with still noted right breast parenchymal thickening and distortion as well as skin thickening.

2. Right internal mammary lymph node measuring 5 mm in diameter.

3. Few millimetric left supraclavicular lymph nodes.

4. Stable few right axillary level II and III lymph nodes including the one inseparable from the pectoral muscles the largest still measures about 14 x 20 mm.

5. Multiple prominent left axillary (level I and II) lymph nodes.

- Still, no sizable pulmonary nodules, cavitary lesions, consolidative patches or bronchiectatic changes.

- No pleural or pericardial collections seen.

- Average cardiac size.

- Still noted bilateral thyroid lobe hypodense nodules with scattered calcific foci.

- Small hiatus hernia.

(II) Abdomen and pelvis:

- Mildly enlarged liver showing homogenous CT density with no focal lesions on non contrast basis.

- Calcular gall bladder.

- No intra or extra hepatic biliary radicals' dilatation.

Print Date: 18/01/2026 03:24:34 PM

Page: 1/2



مركز بهاية للكشف المبكر
وعلاج سرطان الثدي

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شارع بركة أبو حنيفة، القاهرة، مصر 4
شارع بركة أبو حنيفة، القاهرة، مصر 4
DCA, Egypt

Patient Name: FATMA ABD EL AZIZ MOHAMMED A	Gender: Female
MRN: 10124244453	Ordered By: Aya Ahmed Kamal Mohamed
DOB: 18/07/1965 Age: 60 Y 5 M 0 D	Order ID: 3695199

- Bilateral upper calyceal tiny radiodense stones. No renal back pressure changes. Left renal small simple cortical cyst.
- Otherwise, normal non-contrast CT appearance of the spleen, pancreas and suprarenal glands and the rest of the pelvic organs.

- Fibroid uterus seen.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

(III) The examined bone cuts show

- Stationary course of previously noted predominately sclerotic osseous lesions noted at DV5, D8 pedicle, left iliac wing, left 8th rib posterior aspect and left 3rd rib anterior aspect.
- Stationary appearance of left femoral neck comminuted displaced fracture.
- Still noted ventral wedging with structural collapse of D12 vertebral body.
- Dorsolumbar spondylosis degenerative changes with double level L3 and L4 degenerative spondylolisthesis and evident subchondral degenerative changes of L3/4 opposing end plates.

OPINION:

- * Status post follow up showing for a known case of metastatic breast cancer.
- * As correlation was made with the previous non-contrast CT study dated 22/10/2025, the current study shows:

- Mild regression as regard the previously noted few right axillary lymph nodes at level I and few mediastinal lymph nodes.
- Marked regression as regard right supraclavicular and posterior triangle pathologically enlarged lymph nodes.
- Stationary course regarding the rest of the study findings.

Approved by: Elham Mabrouk, MD

Date: 15/01/2026 Time: 09:28

Reported and electronically endorsed by:

Elham ahmed mabrouk hassan

Reported on: 17/01/2026 10:27:00 AM

Patient Name : الأستاذة / فاطمة عبد العزيز محمد عبد النبي	Request No. : 13000288
Patient No. : 26507180101824	Request Date : 26/11/2024
Path No. : SR-1783/24	Verification Date : 28/12/2024
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 59 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data:
Right breast mass.

Nature of specimen:
Block revision.

Gross examination
One paraffin block labelled by (3823/24) was recieved.
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (2+2+2) Grade II.
* DCIS: Could not be detected within the limits of biopsy recieved
* Angioinvasion: Could not be detected within the limits of biopsy recieved.
* IPT:
- ER: Strong nuclear reaction in 90% tumor cells (score 8/8)
- PR: No nuclear reaction in tumor cells (score 0/8)
- Her-2: No membrane reaction in of tumorcells (score 0)
- Ki67: 15% nuclear reaction in tumor cells.

Diagnosis:
Right breast lesion, block revision:
Invasive duct carcinoma, Grade II.
IPT:

Verified by :
dr.esraa magdy

Printing Date : 31/12/2024 09:05:47

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فاطمة عبد العزيز محمد عبد النبي	Request No. : 13000288
Patient No. : 26507180101824	Request Date : 26/11/2024
Path No. : SR-1783/24	Verification Date : 28/12/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 59 Y	Department : Out Patient
Referreing Doc :	Registered by : marwa mohamed youss

Pathology Report

-ER:Positive (score 8/8).
-PR: Negative (score 0/8).
-HER-2:Negative (score 0).
-KI67: 15% nuclear reaction in tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof.Gamal Abdeatty
Dr. Rofaydah Abdallah

Verified by :
dr.esraa magdy

Printing Date : 31/12/2024 09:05:47

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



MDT form

A-Patient ID:

1- Name : فاطمة عبدالعزیز محمد عبدالنبي

Sex:

3- Medical number:

Female

83245698

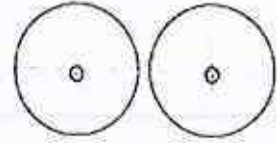
Old Med :

Old Med :

4128-2024

2-ID numbers:

Patient Age : 60 Y



Family history

Family History
-VQ

Phone

Phone

01284060517

BMI

BMI

REASON OF MDT

REASON OF MDT
NEW

marital status:

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

21/01/2025 ☐

Co morbidity

Co Morbidity
dm

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX IDC	ER 8/8	PR 0/8	Her 2 Neu 0	SISH	Ki67 15
BX COMMENT gil	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M bone , LN
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Sonomammography

PATIENT HISTORY

7/2024 denovo MBC to bone , MLN

started faslodex x 4 months

11/2024 PD IN AXILLARY LN -->shifted to AI (BAHYA)

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR IBRANCE + AI + ZOMETA Q 3 MONTH

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. / د. / د. Sh Ha	RADIOLOGY	RADIOLOGY د. / د. / د.
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

PATIENT NAME: FATMA ABD EL AZIZ MOHAMMED ABD EL NABY

GENDER: Female

DATE : 23-Jul-2025 10:08 AM

PATIENT ID: 10124244453

AGE: 60 y

ORDER: 3480174

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 14/11/2024, the current study revealed rather stationary course of the previously seen metastatic lesions at DV5/6, left 8th rib posteriorly, left sacro-iliac joint region, proximal left femur, manubrium sterni and left 3rd rib anteriorly.

Regression of the lesions at DV8 and LV4.

Currently seen active lesion at the right 7th rib posteriorly.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

More or less stationary course of the metastatic osseous deposits, as described.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

23-Jul-2025 10:43 AM

PATIENT: FATMA ABD EL AZIZ MOHAMMED ABD EL NABY, 10124244453, Female, 60 y

Report version 1, 23-Jul-2025 10:43 AM

Page 1 of 1



PATIENT NAME: FATMA ABD EL AZIZ MOHAMMED ABD EL NABY

GENDER: Female

DATE : 17-Jul-2025 11:14 AM

PATIENT ID: 10124244453

AGE: 59 y

ORDER: 3346045,3346046

MULTISLICE NON CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

-Correlation was made with the previous CT study dated 24.04.2025

FINDINGS:

*A known case of breast cancer.

(I) Chest:

• Stationary course regarding:

1. Right breast UOQ mass is seen measuring about 32 x 19 mm along its maximum axial dimensions attached to the overlying skin with still noted right breast parenchymal thickening and distortion as well as skin thickening.
2. Enlarged mediastinal (right para-tracheal and pre-vascular) lymph nodes are seen, the largest currently measures about 15 x 8 mm at the prevascular group.
3. Right internal mammary lymph node measuring 5 mm in diameter.
4. Few millimetric left supraclavicular lymph nodes.

• Size regression of the

1. one of the previously seen pathological right axillary lymph node measuring about 15x 13 mm compared to 18x 20 mm. stable other multiple pathologically enlarged discrete and partly amalgamated right axillary level I, II and III lymph nodes including the one inseparable from the pectoral muscles the largest still measures about 14 x 20 mm.
 2. Right supraclavicular and posterior triangle pathologically enlarged lymph nodes are seen, the largest currently measures about 6 x 2 mm compared to 11x 7 mm previously.
- Still, no sizable pulmonary nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
 - No pleural or pericardial collections seen.
 - Average cardiac size.
 - Still noted bilateral thyroid lobe hypodense nodules with scattered calcific foci.

PATIENT: FATMA ABD EL AZIZ MOHAMMED ABD EL NABY, 10124244453, Female, 59 y

Report version 2, 19-Jul-2025 11:29 AM

Page 1 of 2

- Small hiatus hernia.

(II) Abdomen and pelvis:

- Mildly enlarged liver showing homogenous CT density and enhancement with no focal lesions on non contrast basis.
- Calcular gall bladder.
- No intra or extra hepatic biliary radicles' dilatation.
- Bilateral upper calyceal tiny radiodense stones. No renal back pressure changes.
- Otherwise, normal non-contrast CT appearance of the spleen, pancreas and suprarenal glands.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

(III) The examined bone cuts show

- **Stationary course** of previously noted predominately sclerotic osseous lesions noted at DV5, D8 pedicle, left iliac wing, left 8th rib posterior aspect and left 3rd rib anterior aspect.
- Stationary appearance of left femoral neck comminuted displaced fracture.
- Ventral wedging with structural collapse of D12 vertebral body.
- Dorsolumbar spondylosis degenerative changes with double level L3 and L4 degenerative spondylolisthesis and evident subchondral degenerative changes of L3/4 opposing end plates.

OPINION:

- **Size regression of the**
 1. **one of the enlarged right pathological axillary lymph nodes.**
 2. **Right supraclavicular and posterior triangle pathologically enlarged lymph nodes.**
- **Stationary course regarding the rest of the study findings.**

Reported By,

Dr. Ali Saber, MSc

Dr. Mennatallah Mohamed, MD

19-Jul-2025 11:29 AM

PATIENT: FATMA ABD EL AZIZ MOHAMMED ABD EL NABY, 10124244453, Female, 59 y
Report version 2, 19-Jul-2025 11:29 AM

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مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري . الجيزة

مستشفى بهية الشيخ زايد
ع. الحي الأول - الشيخ زايد أمام كلية الهندسة



STATUS NAME: EATMA ABD EL AZIZ MOHAMMED ABD EL NABY

GENDER: Female

DATE: 26-Jul-2026 10:32 AM

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Technique: Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 5/5/2026, the current study revealed rather stationary course of the metastatic lesions at DV5/6, left 8th rib posteriorly, left sacroiliac joint region, proximal left femur (complicated with fracture), manubrium sterni and left 3rd rib anteriorly as well as right 7th rib posteriorly.

Still noted right tibial bone suggested traumatic lesion. Degenerative changes are seen over dorso-lumbar vertebrae, more advanced at L4.

2447

Changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

Conclusion:
Short term follow up bone scan shows rather stationary course of the metastatic osseous deposits.

Reported By,

Dr. Huda Fathy, MD 26-Jul-2026 03:58 PM

PATIENT NAME: FATIMA ABD EL AZIZ MOHAMMED ABD EL NABY

GENDER: Female

DATE : 11-Aug-2026 09:49 AM

PATIENT ID: 10124244453

AGE: 61 y

ORDER: 3961228,3961229

MULTISLICE NON-CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS REVEALED:*** Status post follow up for a known case of metastatic breast cancer.***** As correlation was made with the previous non-contrast CT study dated 14/04/2026.****FINDINGS:****(I) Chest:**

- No appreciable changes of the previously noted the followings:**

1. Right breast UOQ lesion, still measuring about 32 x 19 mm along its maximum axial dimensions seen attached to the overlying skin with still noted right breast parenchymal thickening and distortion as well as skin thickening.
2. Right axillary lymph nodes, the largest measures 12 x 13 mm.
3. Right internal mammary lymph node measuring 5 mm in diameter.
4. Right supraclavicular and posterior triangle prominent lymph nodes are seen, the largest measures about 5 mm
5. Few millimetric left supraclavicular lymph nodes.
6. Multiple prominent left axillary (level I and II) lymph nodes.
7. Few mediastinal lymph nodes are seen measuring up to 11 x 8 mm at precarinal group.
- Still, no sizable pulmonary nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- Still noted left upper lobe tiny calcified nodule.
- No pleural or pericardial collections seen.
- Average cardiac size.
- Still noted bilateral thyroid lobe hypodense nodules with scattered calcific foci.
- Small hiatus hernia.

(II) Abdomen and pelvis:

- Mildly enlarged liver showing homogenous CT density with no focal

PATIENT: FATIMA ABD EL AZIZ MOHAMMED ABD EL NABY, 10124244453, Female, 61 y

Report version 2, 12-Aug-2026 10:36 AM

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مركز ب.هية التشخيص
أ. د. مناع مكيه - أ. د. خالد النور - أ. د. هادي النور
مستشفى ب.هية التشخيص
1. أ. د. خالد النور - أ. د. مناع مكيه - أ. د. هادي النور

- lesions on non contrast basis.
 - Calcular gall bladder.
 - No intra or extra hepatic biliary radicles' dilatation.
 - Bilateral upper calyceal tiny radiodense stones. No renal back pressure changes. Left renal cortical cyst measures about 2.5 cm.
 - Otherwise, normal non-contrast CT appearance of the spleen, pancreas and suprarenal glands and the rest of the pelvic organs.
 - Fibroid uterus seen.
 - Regular filling of the urinary bladder showing no stones, masses or diverticulae.
 - No abdominal or pelvic lymphadenopathy.
 - No ascites.
 - Vascular atheromatous calcification.
 - Fat containing umbilical hernia.
- (III) The examined bone cuts show stationary course:**
- Multiple predominately sclerotic osseous lesions noted at DV5, D8 pedicle, left iliac wing, left 8th rib posterior aspect and left 3rd rib anterior aspect.
 - Left femoral neck comminuted displaced fracture associated with soft tissue component.
 - Bilateral multilevel rib fractures.
 - Ventral wedging with structural collapse of D12 vertebral body.
 - Dorsolumbar spondylosis degenerative changes with double level L3 and L4 degenerative spondylolisthesis and evident subchondral degenerative changes of L3/4 opposing end plates.

OPINION:

*** Status post follow up for a known case of metastatic breast cancer showing stationary course regarding:**

- *Right breast lesion and right axillary lymph nodes.*
- *Multiple metastatic sclerotic osseous lesions.*
- *No newly developed lesions.*

Reported By,
Dr. Sally Mahmoud, MSc
Dr. Marwa Elsharkawy, MD

12-Aug-2026 10:38 AM

